

Editors
International Journal of Medical Informatics
Elsevier

Dear Editors,

Please consider the enclosed manuscript entitled

*Governing Interruptions After Deterministic Deterioration Scoring:
A Retrospective Evaluation on PhysioNet Challenge 2019*

for publication as an **Original research article** in the *International Journal of Medical Informatics*.

Significance for IJMI. Clinical decision support research often collapses scoring and clinician interruption into one event. We treat interruption policy as a separable, versioned governance layer after a deterministic partial-SOFA score. On the public PhysioNet/Computing in Cardiology Challenge 2019 resource we selected a dual-lane policy on setA, froze it with a content hash, and evaluated setB once. Governed detection matched threshold-only sensitivity (79.5%, 95% CI 77.0–81.8) while the interruptive lane emitted 13.2% as many alerts. Ablations localize the burden reduction to a 90-minute refractory interval and a page gate. The contribution is methodological and operational: reproducible alert-governance evaluation for CDS design, not a claim of clinical outcome improvement.

Article type and fees. Please process this as a subscription (non–open-access) submission. I am an independent, unfunded researcher and cannot pay an article processing charge.

Originality. This manuscript is entirely original. It has not been copyrighted, published, submitted, or accepted for publication elsewhere. No preliminary conference version has been published. Related papers and prior review history: none.

Human subjects and data. No new human-subjects data were collected. The evaluation uses the publicly available, de-identified PhysioNet Challenge 2019 v1.0.0 resource (CC-BY 4.0). The author has no institutional affiliation that requires ethics-board review of this secondary analysis.

Scope note. This is a retrospective offline evaluation of alert routing against a proxy label on a public challenge holdout. It does not report prospective deployment or patient outcomes. A brief Clinical Translation Roadmap is included in the submission package to clarify the intended path from frozen offline evaluation to prospective CDS governance studies.

Conflicts of interest. The author has no conflicts of interest to disclose.

Generative artificial intelligence disclosure. Generative AI tools (OpenAI Codex / GPT-5 and related assistants, accessed 2025–2026) assisted with software implementation for the underlying prototype, manuscript organization, language editing, code review, and L^AT_EX formatting. I directed the work, verified all reported analyses against frozen artifacts, and take full responsibility for the submitted content. These tools are not authors. Reported results use deterministic scoring and rule-based governance; an optional language-model narrative is explicitly outside the alert path and is not part of any reported metric.

Thank you for your consideration.

Sincerely,

Satya Venkata Ranga Janaki Sriram
Mentey, M.S.
Corresponding Author
Independent Researcher
M.S. Computer Science, University of
Missouri–Kansas City
8601 Anderson Mill Rd, Apt 722,
Austin, TX 78729, USA
E-mail: srirammentey@ieee.org
Telephone: +1 (816) 605-8945
ORCID: 0009-0007-2681-006X